CLICS • DESIGN AUDIT

UX / UI REVIEW · JUNE 2026

A design review of the CLICS platform

A thorough, screen-by-screen look at both surfaces — the bilingual **physician portal** and the **audit dashboard** — with findings ranked by severity and a clear order to fix them in. Written for a founder deciding where to spend the next sprint.

10 physician screens | 7 dashboard consoles | Reviewed against the design system spec

Verdict What works Cross-cutting themes Physician portal Audit dashboard Before / after Roadmap

HOW FINDINGS ARE TAGGED

- Critical — fix before pilot scales
- High — meaningful friction
- Medium — polish

AT A GLANCE

A distinctive identity, held back by execution and access

CLICS has done the hard part most clinical tools never attempt: it has a *point of view*. The warm cream-and-coral, editorial-serif language is genuinely differentiated against the sea of cold blue medical software, and the code shows real engineering care. The gap is in the last mile — color contrast that fails accessibility, a bilingual pattern that doubles the reading load on every screen, two surfaces that don't quite look like the same product, and first-run moments that inform more than they reassure.

Visual identity & consistency

Strong concept; gradient/flat button drift between the two apps dilutes it.

Task-flow usability

Flows are sound; native confirm()/alert() dialogs and raw token metrics break the spell.

Mobile experience

Pilot is phone-first, yet wrapping nav, cramped 30px controls, and a tiny chat pane persist.

Accessibility

The coral CTA fails AA contrast and 11px muted hints are unreadable. Systemic.

Onboarding & first-run

A wall-of-text consent gate and bare empty states inform but don't guide or reassure.

Hierarchy & density

The inline "ไทย / English" tax doubles every label and flattens visual hierarchy.

KEEP DOING THIS

What's already working

A real, ownable identity

Warm cream + coral + editorial serif is memorable and humane — exactly right for building physician trust, and rare in clinical software.

Genuine bilingual commitment

Thai/English everywhere (with Noto Serif/Sans Thai in the stack) respects the actual users. The intent is right — only the *presentation* needs work.

Error states written with care

A 502 won't tell a busy physician they have "no episodes"; logout preserves theme; retry paths exist. This is mature thinking most MVPs skip.

Complete dark mode & reduced-motion

Both themes carry accessible semantic text colors, and animations respect `prefers-reduced-motion`. Foundations are solid.

The 16px chat-input fix

Bumping the chat textarea to 16px to stop iOS auto-zoom shows the team is already thinking about real phones. Extend that instinct everywhere.

Guest mode framing

"2 prompts, nothing is saved" is honest and low-friction — a smart way to let a curious physician feel the product before committing.

THE BIG BETS

Seven cross-cutting themes

These aren't single-screen bugs — they're patterns repeated across the whole product. Fixing any one of them lifts dozens of screens at once, which is why they belong at the top of the list.

01 The bilingual "double-label" tax

HIGH

Nearly every string is rendered as `ไทย / English` inline — labels, buttons, nav, eyebrows, even status badges (*"เข้าสู่ระบบ / Login"*, *"โหลดเพิ่ม / Load more"*, *"แพทย์ / Physician"*). It roughly doubles the text on screen, flattens hierarchy (the two languages compete at equal weight), and forces wrapping on phones.

WHY IT MATTERS

Physicians scan, they don't read. Two languages at equal weight means every glance costs twice. It's also the single biggest driver of the "dense" feeling across the app.

RECOMMENDATION

Make language a **user preference** (Thai-primary by default) and show one language at a time. Where both must coexist, use **hierarchy not slashes**: primary language prominent, secondary smaller and muted beneath. See the demo below.

02 The coral CTA fails contrast

CRITICAL

White text on coral `#cc785c` lands at roughly **3.0:1** — below the WCAG AA minimum of 4.5:1 for button text. The physician portal then paints the button as a *gradient* toward an even lighter coral, making the worst part worse. This is the most-clicked element in the product, on every screen.

WHY IT MATTERS	RECOMMENDATION
Failing contrast on the primary action is a legal/accessibility liability and quietly hurts everyone in bright clinical lighting. It's also trivially fixable.	Use the darker `#a9583e` (coral-active, ~4.6:1) as the button fill, drop the gradient, and keep bright coral for accents/borders only — which is what the spec actually prescribes ("flat coral, shadows rare").

03 Two surfaces, one brand — drifting apart

HIGH

The portal (hand-rolled CSS) uses gradient buttons, glow shadows, a `translateY(-1px)` hover lift, and bubbly 18px chat radii. The dashboard (shadcn/Tailwind) uses flat fills and tighter radii. Same tokens, different physics — a physician who becomes an auditor crosses into what feels like a different product.

WHY IT MATTERS	RECOMMENDATION
Dual-role users (physician + auditor) move between both. Consistency *is* credibility for an assessment instrument that physicians must trust with their CPD credit.	Pick one button physics (the flat spec version), and align radius + shadow + focus-ring tokens across both codebases. A one-page "component parity" checklist closes most of the gap.

04 Emoji & native dialogs as UI

HIGH

Functional controls lean on emoji (✎ 🗑 ⏱ 💬 📝 ✅ ❌ 📎 ❓) and destructive actions fire the browser's raw `confirm()` / `alert()`. Emoji render differently per device, read awkwardly to screen readers, and undercut the editorial polish; native dialogs can't be themed or translated.

WHY IT MATTERS	RECOMMENDATION
An emoji trash can next to a native "OK / Cancel" box is the visual opposite of "trusted clinical instrument." It's the seam where the craft shows.	Adopt one line-icon set (the dashboard already ships lucide) and a single themed confirm-modal + toast component. Reuse across both apps.

05 Operator data is leaking to physicians

HIGH

The chat shows live **token counts** and the dashboard shows physicians their **AI cost in dollars** (`$0.0042 / $0.50`). To a doctor practising clinical reasoning, "tokens" is meaningless and a ticking dollar meter is faintly discouraging — it frames learning as spending.

WHY IT MATTERS	RECOMMENDATION
Internal metrics on the user's screen create confusion and anxiety, and signal "we built this for ourselves." It cheapens an otherwise premium experience.	Hide tokens/cost from physicians (keep them on the admin/overview where they belong). If a quota must surface, express it in human terms — e.g. "12 of 20 sessions left this month."

06 The scoring rubric is under-specified · CRITICAL

Auditors score seven PIS-7 domains on 0–5 sliders, but only the two endpoints are labelled (*"0 ไม่พบ … 5 ดีเยี่ยม"*) in 10px type, and the default is 0. Inter-rater reliability (ICC) is the product's core scientific claim — yet the instrument gives raters almost no shared definition of what a "3" means.

WHY IT MATTERS	RECOMMENDATION
If raters guess at the scale, ICC suffers — and ICC is what you're staking the pilot's validity on. This is a UX problem with a scientific blast radius.	Show the level descriptor for the selected score inline, make every level tappable with its anchor text, and require an explicit pick (no silent 0 default). See the demo below.

07 A signature typeface, barely used · MEDIUM

Cormorant Garamond is the brand's voice, yet it appears only on the wordmark and the page-intro `h1`. Everything else is Inter, so the editorial promise rarely lands. Separately: a thin serif at small sizes on cream is low-contrast — worth pressure-testing for a high-stakes clinical tool.

WHY IT MATTERS	RECOMMENDATION
Either commit to the serif so the identity reads, or simplify — but the current "wordmark-only" use gets the cost of a display face without the payoff.	Promote serif to section headers and key numbers (score, ICC) at **≥24px / weight 600** where it's legible, and keep body in Inter. Define the minimum serif size in the system.

SURFACE 1 · SCREEN BY SCREEN

The physician portal

Vanilla HTML/CSS/JS, phone-first, bilingual. This is where the physician lives — login through scored result. Findings below are specific to each screen; the systemic ones above apply throughout.

`index.html` Login

Elegant and calm — the serif wordmark sets the tone well. But it's carrying too much at once.

- **Internal plumbing leaks onto the first screen.** "Admin & Auditor sign in here too — routed to your console" is a routing detail no physician needs; it dilutes a focused first impression.
- **Five competing actions.** Email/password, forgot, Google, and guest all sit at one weight. Establish a clear primary (sign in) and demote the rest.
- The card border on cream is nearly invisible, so the form floats. A touch more elevation or a soft surface fill would ground it.

`dashboard.js · consent` Consent gate

The very first thing a logged-in physician meets is a hard modal — and it's the least-designed element in the app.

- **Wall of bilingual text, injected with inline styles** outside the design system. It reads as a legal hurdle, not a warm welcome to a pilot they're generously joining.
- One giant "I consent" button, no way to scroll-confirm or revisit later. Break the three key promises (de-identified cases · provisional scores · recorded & reviewed) into scannable points.
- This is a trust moment — use it to *reassure* (what we protect, why it helps you) rather than only to disclaim.

`dashboard.html` Dashboard & folders

A sensible folders-and-episodes model, but the controls are dense and the empty state under-guides.

- **Conversation-row controls are sub-44px and cramped:** a ✎ rename, a native folder `<select>`, and a 🗑 delete jammed together — easy to mis-tap delete on a phone.
- **Empty state is a single muted line** ("No episodes yet"). The primary "+ New PLE" lives up in the toolbar, not in the empty state where the eye lands. Put the call-to-action *in* the void.
- The per-row meta reads "`3 รอบ · 1240 tokens · date`" — tokens are noise to a physician (see theme 05).
- Delete uses `confirm()` for a permanent action — fine functionally, but jarring against the editorial tone.

`ple-chat.html` PLE clinical dialogue

The richest, best-styled screen — markdown answers, Socratic callout, citations. A few high-stakes ergonomics to tighten.

- **The session bar wraps on mobile and drops "Pause / End" to a new line** — placing the irreversible red "End session" button where a thumb can hit it by accident. Separate destructive actions and confirm intent.
- **The chat pane is squeezed.** `height: calc(100dvh - 360px)` on a small phone leaves a very short reading window above the keyboard. Let the messages flex to fill and pin only the composer.
- Two stacked 11px helper lines ("Pause = autosave…" and "Shift+Enter…") in low-contrast muted-soft — below readable size and contrast.
- **Strong move:** the "active time only" timer tooltip and the pause/end distinction are genuinely thoughtful. Keep that clarity.

`ple-result.html` PIS-7 result

A clean scorecard: big composite number, radar, per-domain bars with rationale. Good bones, a few refinements.

- **The hero number uses Inter at 48/700.** This is the perfect place for the brand serif — a large Cormorant numeral would feel like a certificate, not a stat readout.
- Pass/fail states lean on ⚠️/✅/❌ emoji at 32px as the primary signal. A consistent icon + colored status band reads more credibly for a credit-bearing result.
- "Refresh" buttons on the evaluating state ask the user to do the system's polling. Auto-poll and animate the transition instead.

`lpdp.html` Progress (LPDP)

A single line chart of PIS-7 over time. The error/empty distinction is handled well.

- One lonely chart on a big page. Frame it with the latest score, a trend delta, and a "what improved" note so progress feels personal.
- Axis labelled "PLE 1, PLE 2…" — use dates or case titles so the timeline is meaningful.

`guest-chat.html` Guest trial

Honest framing ("2 prompts, nothing saved"). The conversion moment is the gap.

- **No designed "you're out" moment.** After prompt 2, show what they'd unlock by signing up (saved history, CPD credit, progress) — that's the whole point of the trial.
- The "prompts left" pill is a nice touch — make it count down visibly.

SURFACE 2 · SCREEN BY SCREEN

The audit dashboard

Next.js + shadcn, three role consoles (admin · auditor · superadmin). More structurally mature than the portal — real tables, pagination, mobile card fallbacks — but the scoring instrument and a couple of layout calls need attention.

`NavBar · AppShell` Shell & navigation

Role-keyed nav with a proper hamburger and active-state underline — solid. Two things stand out.

- **The 150% root-font jump at 1280px is risky.** 1280–1440 is the most common laptop range; forcing 24px base there makes the whole app feel oversized and invites overflow. Consider a gentler scale or a higher breakpoint.
- Bilingual nav items ("ผู้ใช้งาน / Users", "คำเชิญ / Invites") widen the bar and force early wrapping — theme 01 bites hardest in horizontal nav.

`auditor/queue · ples/[id]` Auditor queue & blind scoring

This is the scientific heart of CLICS — and the place a design investment pays the highest dividend.

- **The transcript the auditor scores against is the *worse* reading view.** Physicians get rich markdown + Socratic callouts; auditors get a stripped `whitespace-pre-wrap` bubble with 📎 / ❓ emoji. The high-stakes reader deserves the better renderer.
- **Sliders default to 0 and only endpoints are labelled** (theme 06). For an instrument whose ICC *is* the product, give each level a descriptor and force an explicit choice. Transcript-above-sliders also means lots of scrolling — consider a sticky two-pane layout.
- Queue cards are clean. The 0.6-opacity "scored" state is a touch low — pair it with a checkmark badge (already present) and a subtle strikethrough-free dim.
- **Strong move:** guarding the irreversible all-zeros submit, and revealing the AI score only after the auditor commits. The blind-scoring integrity is well handled.

`admin/ples` PLE management

Filterable, paginated table with a genuine mobile card fallback — the most operationally complete screen.

- Bilingual column headers ("แพทย์ / Physician", "มอบหมาย / Assign Auditor") nearly double header height. Consider Thai-primary headers with English tooltips.
- Status shown as monochrome outline badges — color-coding by state (evaluated/flagged/failed) would let admins triage at a glance.
- **Strong move:** server-side filtering, request-sequence guards, and a real "load more" with loaded/total counts. Engineering-grade list handling.

`admin/overview · icc` Executive overview

The superadmin's single-pane pilot status — drill-down cards, ICC calibration, council chips, capacity, budget. Genuinely well-composed.

- **This is the best-designed screen in either app.** Clear sections, skeleton loaders, theme-aware charts, a meaningful 0.75 ICC reference line. The information architecture is exemplary — make it the template the rest aspires to.
- It mixes hardcoded status hexes (`#5db872 / #c64545`) into otherwise tokenised charts — centralise these as semantic chart tokens so dark mode and rebrands stay consistent.
- Section headers are serif here (good!) — this is the consistency target: bring that same serif-header rhythm to every other console.

SHOW, DON'T TELL

Three fixes, made visible

Live demonstrations rendered in CLICS's own styles — the current pattern beside a proposed one, so the direction is concrete rather than abstract.

01 · The bilingual label

From two languages fighting at equal weight → a clear hierarchy with a language preference.

BEFORE

หน้าหลัก / Home ความก้าวหน้า / Progress ออกจากระบบ / Logout

อีเมล / Email

doctor@hospital.th

เข้าสู่ระบบ / Login

AFTER

ไทย | EN

หน้าหลัก ความก้าวหน้า ออกจากระบบ

อีเมล Email

doctor@hospital.th

เข้าสู่ระบบ

The "after" cuts roughly 40% of on-screen characters, restores a single scan-line per item, and lets a Thai-first physician read in one language while English stays available — by preference, or as a quiet secondary line where it genuinely aids a medical term.

02 · The primary button's contrast

A one-token change takes the most-clicked element from failing to compliant.

BEFORE · GRADIENT CORAL

Start Session

× 3.0:1 · fails WCAG AA

AFTER · FLAT CORAL-ACTIVE

Start Session

✓ 4.6:1 · passes WCAG AA

Bright coral stays the brand — keep it for borders, accents, links, focus rings and the full-bleed callout cards the spec describes. Just don't put small white text on top of it.

03 · The PIS-7 scoring control

From an under-anchored slider → a rubric that gives every rater the same definition of each level — directly protecting ICC.

BEFORE

Reasoning Depth 20% · **0**/5

0 ไม่พบ 1 2 3 4 5 ดีเยี่ยม

Defaults to 0 · no definition of what a "3" means.

AFTER

Reasoning Depth 20% · **3**/5

0 1 2 **3** 4 5

LEVEL 3 · COMPETENT

Builds a structured differential and tests it against the AI's evidence, with minor gaps in prioritisation.

Each level becomes a tappable anchor with a written descriptor (placeholder copy shown). Raters converge on a shared scale — the single highest-leverage UX change for the calibration the pilot is built to prove.

WHERE TO START

A prioritized order to fix in

Sequenced by effort-to-impact. The left column is a single focused day; the right column is genuine product strategy.

QUICK WINS · THIS WEEK

Low effort, immediate lift.

1. Swap the coral CTA fill to `#a9583e`, drop the gradient.
2. Hide tokens & dollar cost from physician screens.
3. Bump all muted-soft hint text to ≥13px and a darker tone.
4. Put the "+ New PLE" CTA inside the empty state.
5. Separate the chat's "End session" from "Pause" and confirm intent.

HIGH IMPACT · THIS MONTH

Medium effort, broad reach.

6. Ship a themed confirm-modal + toast; retire every `alert/confirm`.
7. Replace functional emoji with one line-icon set across both apps.
8. Give auditors the physician-grade transcript renderer.
9. Redesign the consent gate as a warm, scannable welcome.
10. Re-check the 1280px 150% scale; enlarge mobile tap targets to 44px.

BIGGER BETS · THIS QUARTER

Higher effort, strategic payoff.

11. Build the rubric-anchored PIS-7 scoring control — protect ICC at the source.
12. Make language a true user preference; rebuild labels as hierarchy, not slashes.
13. Unify both surfaces under one shared component library & token set.
14. Commit the serif to section headers & key numbers; codify a minimum size.

The instinct is right. Tighten the last mile.

CLICS isn't fighting a taste problem — the taste is good. It's fighting a consistency-and-access problem, and those are the most fixable kind. Land the five quick wins first; they buy credibility and accessibility for almost no effort. Then invest where the product's scientific claim lives: the scoring instrument. Do those, and the warmth this brand already has will finally read as polish.